Journal of Meaning-Centered Psychology
PUBLISHED BY THE INSTITUTE FOR MEANING-CENTERED PSYCHOLOGY

Volume 1 · Issue 3 · October 2026
pp. 40–43 · Open access · CC BY 4.0

THEORETICAL ARTICLE

A Working Alliance with a Machine?

Bordin's goals, tasks and bond and the limits of relationship with conversational software

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ABSTRACT

The working alliance is among the most consistently studied correlates of psychotherapy outcome, and researchers of mental health chatbots increasingly report that users form an alliance with their software. This theoretical contribution examines whether, and in what sense, such a claim is coherent. Starting from Bordin's pantheoretical formulation of the alliance as agreement on goals, agreement on tasks and an affective bond, we argue that the first two components can be partly realized in human–software interaction, because they concern structure, transparency and collaboration. The bond component is more problematic. Self-report studies with conversational agents such as Woebot and Wysa suggest that users describe feelings of being understood and supported, yet these reports are also compatible with the ELIZA effect, the human tendency to attribute understanding to systems that merely simulate responsiveness. Drawing on Buber's distinction between I–Thou and I–It relations and on Frankl's view of therapy as personal encounter, we distinguish an experienced bond from a reciprocal bond. We propose that a "digital working alliance" should be conceptualized as a user-side construct, measured with validated instruments and interpreted cautiously. Implications for research, clinical practice and transparency are discussed.

Keywords: working alliance · conversational agents · ELIZA effect · therapeutic relationship · logotherapy

1 Introduction

Few concepts in psychotherapy research have attracted as much empirical attention as the working alliance. Meta-analytic syntheses report a robust, moderate association between the quality of the alliance and treatment outcome across modalities, formats and patient populations (Flückiger et al., 2018), and the alliance features prominently in contextual accounts of why psychotherapy works (Wampold & Imel, 2015). It is therefore unsurprising that researchers and developers of conversational agents for mental health have begun to ask whether users form an alliance with software.

Early evidence appears to answer this question in the affirmative. Users of the cognitive-behavioral chatbot Woebot reported bond scores on an adapted alliance measure that the authors described as comparable to those observed in human-delivered therapy (Darcy et al., 2021), and similar findings have been reported for Wysa (Beatty et al., 2022). In a randomized trial of a generative AI chatbot, participants likewise reportedly rated the therapeutic alliance favorably (Heinz et al., 2025).

This article asks a prior, conceptual question: what would it mean to have a working alliance with a machine? We revisit Bordin's (1979) formulation, examine which of its components can be meaningfully transferred to human–software interaction, discuss the ELIZA effect as a rival explanation for reported bonds, and introduce an existential perspective informed by Buber and Frankl. We conclude with proposals for how a digital working alliance should be conceptualized and studied.

2 Bordin's pantheoretical concept of the alliance

Bordin (1979) proposed that the working alliance is a pantheoretical construct, relevant to every form of change-inducing relationship. He defined it through three components: agreement between patient and therapist on the goals of treatment, agreement on the tasks through which these goals are to be pursued, and the development of a bond of trust, liking and attachment. Different therapies, he argued, place

different demands on each component, but none can dispense with them altogether.

Bordin's formulation proved highly productive. It informed the Working Alliance Inventory (Horvath & Greenberg, 1989), which became one of the most widely used alliance measures and which operationalizes goals, tasks and bond as separate subscales. Subsequent work emphasized that the alliance is a process that is repeatedly negotiated, ruptured and repaired rather than a static property of the relationship (Safran & Muran, 2000).

Two features of this tradition matter for the present question. First, the alliance is defined as collaborative and reciprocal: both partners contribute to it and both are affected by it. Second, the alliance has largely been measured through self-report, often from the patient's perspective alone. The second feature makes it possible to administer alliance questionnaires to chatbot users; the first raises the question of whether what such questionnaires capture still corresponds to Bordin's construct.

3 Goals and tasks: what software can share

The goal and task components translate to digital settings more readily than the bond. A structured program can state its aims explicitly, invite users to select or prioritize goals, and explain the rationale for exercises such as mood monitoring, cognitive restructuring or behavioral activation. Agreement on goals and tasks in this sense depends primarily on transparency, plausibility and fit, all of which can, at least in part, be designed into software.

Internet-delivered interventions illustrate this point. Guided programs have accumulated substantial evidence of efficacy for several conditions (Andersson et al., 2019), and their effects plausibly depend in part on users accepting the rationale and engaging with the tasks. A conversational interface may strengthen such agreement by adapting explanations to user questions, checking understanding and responding to stated preferences, functions that an early review of the alliance in digital mental health identified as promising but under-studied (Henson et al., 2019).

Even here, however, an asymmetry remains. In human therapy, agreement on goals results from negotiation between two parties who both hold views about what is good for the patient. Software holds no such views; its goals are those of its designers, encoded in advance. What the user experiences as agreement is better described as acceptance of a predefined framework. This may be a legitimate therapeutic ingredient, but it is not mutual agreement in Bordin's sense.

4 The bond problem and the ELIZA effect

The bond component poses the central difficulty. Bordin (1979) described it in terms of trust, liking and attachment between two persons. Self-report data indicate that some users do experience something resembling such a bond toward chatbots (Beatty et al., 2022; Darcy et al., 2021). The question is how these experiences should be interpreted.

Weizenbaum (1966) observed that users of ELIZA, a simple pattern-matching program that imitated a Rogerian therapist, readily attributed understanding to it. The tendency to treat systems that produce socially contingent responses as if they were social partners has since been documented in many contexts (Reeves & Nass, 1996), and Turkle (2011) argued that such technologies offer the feeling of companionship without the demands of reciprocity. Large language models produce fluent, context-sensitive and empathic-sounding text, which makes such attributions considerably more likely than they were with ELIZA.

From this perspective, a high bond score on an adapted alliance measure is ambiguous. It may indicate a therapeutically useful experience of being heard, which could support engagement and hope. It may also indicate an anthropomorphic projection that overestimates the system's capacities, with potential risks if users rely on it in crises or disclose more than they otherwise would (Fiske et al., 2019). Questionnaire scores alone cannot distinguish between these interpretations.

5 Encounter and reciprocity: an existential perspective

Buber (1923/1970) distinguished the I–It relation, in which the other is experienced as an object of use or knowledge, from the I–Thou relation, in which persons meet in mutual presence. The I–Thou relation is not a feeling inside one person but something that happens between persons. On this account, a bond in the full sense requires that both partners are addressed and can address the other in return.

Frankl similarly understood psychotherapy as a personal encounter in which the therapist meets the patient as a spiritual person capable of responsibility and meaning, not merely as a mechanism to be repaired (Frankl, 1986). The therapist's own commitment, conscience and capacity to be affected belong to this encounter. Software can simulate the

verbal surface of such engagement, but it cannot be moved, held accountable or present in this sense.

This does not imply that interactions with software are worthless. People find insight, structure and comfort in many non-reciprocal activities, such as reading, journaling or reflective writing. The existential perspective suggests, however, that conversations with an AI system belong closer to this category of resources than to the category of personal encounters, regardless of how the user experiences them in the moment.

6 Toward a careful concept of the digital working alliance

We therefore propose distinguishing an experienced bond, a user's subjective sense of being understood and supported, from a reciprocal bond, a relation in which both partners hold commitments and are accountable to one another. A digital working alliance can include acceptance of goals and tasks and an experienced bond, but it lacks a reciprocal bond. The construct should accordingly be framed as a user-side measure of engagement with and trust in an intervention rather than as equivalent to the therapeutic alliance.

Several implications follow. Adapted instruments should be validated for this context rather than assumed to be equivalent, and comparisons with norms from human therapy should be made with caution. Studies should examine whether experienced bonds predict outcomes, adherence or adverse events, and whether transparency about the system's nature changes alliance ratings. For practice, honest communication that users are interacting with software appears ethically required and may protect against misplaced reliance (Fiske et al., 2019).

7 Conclusion

Bordin's framework helps clarify what conversational agents can and cannot offer. Goals and tasks can be partly designed into software, and an experienced bond can arise; a reciprocal bond cannot. Recognizing this difference allows researchers to take user experiences seriously without equating them with personal encounter. It also reminds practitioners that the human relationship in psychotherapy is not simply one delivery channel among others but a distinct way of being with another person, one in which meaning is discovered together rather than supplied.

For meaning-centered psychology, the question is therefore not whether software should be excluded from psychological care, but how it can be positioned honestly. Conversational agents may serve as structured aids between sessions, as low-threshold entry points or as tools for self-reflection. Their value is likely to be greatest when users understand what they are dealing with, and when such systems point beyond themselves toward the human relationships, tasks and responsibilities in which a person's life can become meaningful.

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HOW TO CITE

Ashford, S. R., Pazer, S., & Whitcombe, E. M. (2026). A Working Alliance with a Machine?: Bordin's goals, tasks and bond and the limits of relationship with conversational software. Journal of Meaning-Centered Psychology, 1(3), 40–43.

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